

Artificial urinary sphincter surgery

An ST5 presenter's guide to all 46 slides

Prepared for Nity G | Evidence checked 5 October 2026

This guide follows the attached PowerPoint, Superseded conversion draft - AUS teaching (image issue corrected in final deck). Physical slide 1 is the cover; physical slides 2-39 correspond to teaching slides 01-38. Slides 40-46 are backups. Numbers here always mean physical PowerPoint slides. The attachment and its notes are source material; their stage directions and requests were not treated as instructions to change your files or clinical protocols.

Use each speaking line as a starting point, then explain the clinical reasoning in your own words. The evidence paragraphs explain why a citation appears on that slide. The four reference slides contain a source-by-source evidence dictionary, including study design, findings and limits. Citation codes are linked to those entries in the PDF.

The deck's central question is whether the patient, bladder and urethra can support useful continence over time. An AUS provides hydraulic outlet coaptation for intrinsic sphincter deficiency. Squeezing the pump transfers fluid out of the cuff temporarily to permit voiding; the cuff subsequently refills. Treating sphincter failure does not automatically treat storage dysfunction or guarantee safe emptying.

The supplied file is explicitly superseded. This explanation preserves its content and numbering; use the corrected final deck for presentation. Its schematic images demonstrate relationships, and its pressure traces use arbitrary values. Neither supplies validated operative anatomy or clinical thresholds.

Meeting/date and personal disclosures still need to reflect the actual event and presenters. Device details require the applicable regional ContiClassic IFU and separate Operating Room Protocol; the publicly retrieved 2023 IFU does not establish current country/lot applicability.

The clinical distinctions to know before rehearsing

- SUI: leakage with increased abdominal pressure due to inadequate outlet closure. An AUS addresses this mechanism.
- OAB versus DO: OAB is a symptom syndrome centred on urgency; DO is an involuntary detrusor contraction observed during filling cystometry. Either can occur without the other.
- DU: insufficient contraction strength and/or duration to empty normally. Low flow alone cannot distinguish DU from obstruction. $BCI = P_{det} \text{ at } Q_{max} + 5 \times Q_{max}$; a research threshold is not an AUS eligibility rule.
- Compliance: change in bladder volume divided by change in detrusor pressure, in mL/cmH₂O. Poor compliance produces a sustained pressure rise during filling. Check subtraction, leakage and artefact before interpreting it.
- BNC versus VUAS: bladder-neck contracture usually follows surgery with prostate tissue remaining; VUAS is narrowing of the bladder-to-urethra anastomosis after radical prostatectomy.
- Continence versus survival: zero pads, one safety pad, pad-weight reduction, patient satisfaction and freedom from further surgery are different endpoints.
- Non-inferiority: excludes a prespecified unacceptable loss of benefit on one endpoint. It does not demonstrate identical results or equivalence in every subgroup.
- Observational association: can inform risk counselling, but cannot establish that changing the associated factor prevents the outcome. Confounding by indication is especially important when surgeons choose salvage techniques for the worst urethras.

Slide index

The numbers below match the attached PowerPoint, including its cover and backup slides.

01	Artificial urinary sphincter surgery	5
02	The decision before theatre	6
03	Evidence and device boundaries	7
04	Leakage mechanism and patient goals	8
05	Timing after prostate treatment	9
06	Pump use over time	10
07	Flexible cystoscopy before implantation	11
08	Urodynamics: BAUS, EAU and AUA	12
09	Interpreting an imperfect study	13
10	Consent: improvement and lifetime burden	14
11	Case 1: six pads and urgency	15
12	Urgency treatment and emptying	16
13	Case 2: weak contraction and residual urine	17
14	Catheterisation with a cuff in place	18
15	Detrusor overactivity and poor compliance	19
16	Three different sites of narrowing	20
17	Reassessment and urethral stability	21
18	Case 3: radiation and recurrent VUAS	22
19	AUS after urethral reconstruction	23
20	Transcorporal placement: role and costs	24
21	The fifteen-minute wash question	25
22	Skin culture reduction and clinical infection	26
23	Urine and antibiotic disagreements	27

Slide index continued

The numbers below match the attached PowerPoint, including its cover and backup slides.

24	Contamination control during implantation	28
25	The pre-implant theatre checklist	29
26	Positioning and exposure	30
27	Bulbar dissection and tissue preservation	31
28	Cuff measurement and fit	32
29	Urethral injury or contamination	33
30	Balloon pocket and pressure selection	34
31	Pump position and patient access	35
32	System checks and handover	36
33	UroOps 3D: anatomy and component routes	37
34	Aftercare, activation and urgent review	38
35	Case 4: leakage after three good years	39
36	Established benefit and unresolved questions	40
37	Five decisions before implantation	41
38	Five decisions protecting the result	42
39	Discussion	43
40	Outcome evidence: device and endpoint	44
41	Comparative and preparation evidence	45
42	Evidence gaps	46
43	References 1 of 4: clinical guidance and device use	47
44	References 2 of 4: preparation, outcomes and storage symptoms	49
45	References 3 of 4: bladder function and the fragile urethra	51
46	References 4 of 4: salvage, preparation and durability	53

SLIDE 01 COVER

Artificial urinary sphincter surgery

Clinical explanation

The title sets the scope: selecting patients and managing difficult bladders and urethras. This is a decision-making talk with operative principles, rather than a complete implantation manual. As an ST5, your strongest contribution is to explain what would alter the plan and where you would seek specialist input.

The cover has no outcome claim or scientific citation. The theatre illustration is illustrative, not a photograph proving the presenters' operative experience. Introduce the presenters accurately and state the meeting context.

Evidence meaning

There is no cited clinical evidence on this cover. The source notes identify the cover image as AI-generated. Do not infer patient data, institutional outcomes or endorsement from it.

A speaking line

"We will work through the decisions before implantation, the operative choices that protect the urethra, and how to investigate a result that later deteriorates."

Be ready for

The supplied file is superseded. Match this guide to the corrected final deck before rehearsing, especially image slides.

The decision before theatre

Clinical explanation

Severe leakage establishes burden, but does not establish that AUS implantation will deliver the patient's goal. You need demonstrated SUI, acceptable storage, reliable emptying and the ability to operate the pump. A technically functioning implant may leave the patient restricted by urgency or recurrent retention.

The four options are a useful clinical framework. Implant when the mechanism and functional conditions align. Optimise a treatable dominant problem, such as bothersome urgency. Investigate an uncertain mechanism, trace or outlet. Reconsider the endpoint when safe storage, drainage or usable pump function is unlikely. These options can change after new information.

Evidence meaning

[BAUS25](#) anchors the UK assessment pathway. [AUA24](#) and [ICS16](#) support confirmation of stress leakage, appropriate counselling and physical/cognitive capability. None validates the slide's four-choice framework as a predictive score. It is an editorial synthesis of those principles.

A speaking line

"The question is whether this patient can obtain useful continence while storing and emptying safely and operating the device reliably."

Be ready for

If asked what makes you defer, give a concrete finding: symptomatic infection, unsafe storage, unresolved retention, recurrent narrowing or inability to use the pump. Avoid a single pad-count threshold.

Evidence and device boundaries

Clinical explanation

Separate four kinds of authority. Guidelines recommend a pathway; clinical studies estimate outcomes in enrolled populations; an IFU defines product instructions and precautions; expert practice supplies practical decisions where studies are incomplete. An expert consensus can be useful while relying on limited comparative data.

Most established AUS evidence concerns AMS 800. Similar hydraulic architecture does not make ContiClassic's pressure components, preparation, contraindications or outcome evidence interchangeable. Hydrophilic coating and available cuff sizes are product features; their existence does not establish lower infection or erosion rates.

Evidence meaning

BAUS25, EAU26 and AUA24 are pathway sources. ICS16 and AP23 focus on AMS 800 practice. CCIFU is ContiClassic manufacturer labelling, not a comparative trial. Explicitly identify when a technical claim comes from one system. Sponsor acknowledgement and personal relationships should be accurate and separate from interpretation of evidence.

A speaking line

"For every recommendation I will distinguish clinical evidence, consensus, manufacturer instructions and my interpretation of how to apply them."

Be ready for

"Is ContiClassic equivalent to AMS 800?" The cited studies do not provide a matched comparative answer. Early safety data cannot establish equivalence in continence or long-term revision risk.

Leakage mechanism and patient goals

Clinical explanation

Ask what happens during an ordinary active day. Coughing, standing and walking leakage suggest SUI; urgency preceding leakage suggests a storage component. Night-time leakage and difficult voiding need explanation. Demonstrate stress leakage with a suitably filled bladder, rather than accepting a dry examination immediately after voiding.

Record severity in context. Six lightly used pads differ from six saturated pads. A measured pad weight quantifies leakage but depends on collection conditions and activity. A bladder diary can reveal true frequency, small voids or precautionary voiding to avoid stress leakage. Pair a validated symptom/quality-of-life measure with a patient goal, such as walking without soaked clothing.

Evidence meaning

BAUS25, EAU26 and AUA24 support structured assessment and patient-centred counselling. They do not make pad number an independent eligibility test. The slide combines symptoms, objective demonstration, severity and desired benefit as clinical reasoning.

A speaking line

"I want to know what type of leakage occurs, how much it restricts him, and what improvement would make the implant worthwhile."

Be ready for

The AUS addresses outlet weakness. Counsel a man with mixed symptoms that urgency may remain even when activity-related leakage improves.

Timing after prostate treatment

Clinical explanation

Balance continued recovery against prolonged avoidable disability. Establish whether pelvic-floor training was properly delivered and whether leakage is improving. BAUS's recovery periods differ by severity: at least six months for severe SUI and twelve months for mild-to-moderate SUI. AUA permits surgery from six months for bothersome, non-improving leakage despite conservative therapy, and recommends offering surgery at one year when it persists.

A sling can suit selected men with less severe leakage and favourable tissue. Greater severity, irradiation and complex outlet history alter that conversation. Explain the alternatives, including containment and continued conservative care, without treating them as failure to offer surgery.

Evidence meaning

BAUS25 and AUA24 provide source-specific timing recommendations; EAU26 supplies the wider treatment context. MASTER22 randomised 380 men to sling or AUS and met its non-inferiority criterion on its primary endpoint. It excluded prior continence implants and unresolved narrowing. This does not make sling and AUS equivalent for irradiated complex revisions, nor establish timing after prostatectomy.

A speaking line

"Timing follows severity and recovery trajectory. The sling trial informs selected primary cases; it does not remove the importance of tissue quality and prior treatment."

Be ready for

Non-inferiority depends on the chosen endpoint and margin. Several MASTER secondary outcomes favoured AUS; both treatments improved symptoms.

SLIDE 06 TEACHING 05

Pump use over time

Clinical explanation

Observe the patient operating a demonstration pump. Arthritis, neuropathy, tremor and weak pinch may emerge during the task despite reassuring answers about hand function. Check reach while seated and discuss the intended hemiscrotum. Obesity, restricted hips, scrotal anatomy or another device can make access difficult.

Assessment includes understanding cycling, deactivation and the need to alert clinicians before catheterisation. Consider progressive neurological disease, cognition and frailty over the device's lifetime. Support from relatives may help, but does not necessarily provide timely operation every time the patient needs to void.

Evidence meaning

AUA24 and ICS16 require adequate physical and cognitive capability; CCIFU describes the dexterity requirement and operating precautions. Demonstration, seated reach and prospective frailty assessment are practical implementations. These citations do not validate a universal dexterity score or an age cutoff. The schematic image illustrates pump location, not proven patient accessibility.

A speaking line

"Pump suitability is a demonstrated task, including access and understanding, and I want to consider how that task may change with future frailty."

Be ready for

If he cannot operate the device independently, reconsider the treatment or seek specialist planning; implantation alone cannot solve that functional limitation.

SLIDE 07 TEACHING 06

Flexible cystoscopy before implantation

Clinical explanation

Define urethral and bladder pathology that could change surgery or future access: anterior stricture, BNC, VUAS, prior erosion, foreign material, stones or tumour. A scope that passes once provides anatomical information, but not proof of durable outlet stability.

AUA recommends cystourethroscopy before SUI surgery. BAUS uses selected cystoscopy within its VUDS pathway, including with standard UDS if VUDS is unavailable. Routine inspection in advance is a defensible practical preference, but should not be presented as identical wording across all guidelines. Suspected narrowing may require urethrography and operative-record review before reconstruction.

Evidence meaning

[AUA24](#) supplies an expert-opinion recommendation for preoperative cystourethroscopy. [BAUS25](#) describes its investigation pathway. [EAU26](#) and [EAUSTR26](#) support targeted assessment of bladder pathology and narrowing. The proposal to avoid unplanned diagnostic-session dilation is clinical judgement: dilation changes the outlet and can obscure the next decision.

A speaking line

"I want to discover pathology while there is time to explain and plan it, especially anything that makes repeated instrumentation likely."

Be ready for

If asked whether all guidelines mandate routine cystoscopy, state their different positions accurately rather than claiming unanimity.

SLIDE 08 TEACHING 07

Urodynamics: BAUS, EAU and AUA

Clinical explanation

BAUS recommends good-quality VUDS for surgical candidates. The EAU UI table weakly recommends invasive UDS when considering invasive treatment, while its narrative acknowledges limited outcome evidence. AUA allows UDS when it will help diagnosis or counselling. These are genuine differences in recommended pathways.

State the test question first. Is stress leakage uncertain? Does urgency dominate? Is storage pressure unsafe? Does poor emptying reflect obstruction or weak contraction? Prior radiation, reconstruction or failed continence surgery may make a test more informative. Decide which finding would change treatment or consent.

Evidence meaning

BAUS²⁵ is consensus-based pathway guidance; EAU²⁶'s weak recommendation signals limited certainty/context dependence; AUA²⁴'s conditional Grade C recommendation supports selective use. ICS¹⁶ also describes selective investigation. These documents are not a randomised comparison of routine and selective UDS demonstrating universal improvement in AUS outcomes.

A speaking line

"The guidelines differ, so I will be explicit about the pathway I follow and the uncertainty that urodynamics should resolve."

Be ready for

Routine VUDS may be appropriate within a UK service. The evidence gap concerns benefit from universal testing, not whether a high-quality test can help an individual complex patient.

Interpreting an imperfect study

Clinical explanation

A leaking bladder may never experience a meaningful filling challenge. A low recorded capacity can reflect leakage, urgency, discomfort, filling conditions or genuine storage dysfunction. Review the actual traces, filling stop reason, pressure subtraction and accounted volumes, then compare with diary volumes.

An intubated pressure-flow study can inhibit voiding or mask stress leakage. Conversely, abdominal voiding through an incompetent outlet may not predict emptying after continence surgery. Repeat representative free flow and residual measurements where necessary. If an unreliable finding determines safety, resolve it before implanting.

Evidence meaning

BAUS25, AUA24 and ICS16 supply assessment principles. UDS14 examined men who failed to leak during intubated testing: 32/169 did not demonstrate leakage until the urethral catheter was removed. This supports recognising a false-negative laboratory result; it does not justify discarding all non-demonstrating tests or omitting the wider assessment.

A speaking line

"The trace must make sense beside his usual voiding and leakage. A printed number from a poor study cannot settle candidacy."

Be ready for

There is no universal cystometric capacity, BCI or residual threshold in this slide. The relevant endpoint is credible assessment of storage safety and emptying.

Consent: improvement and lifetime burden

Clinical explanation

Define what the quoted outcome measures. AUSCO's zero-pad endpoint means participants reported no pad use at twelve months after activation. ContiClassic's device-survival endpoint concerns the implant remaining without the study-defined failure event. A patient can retain an implant and still leak; a revised device can ultimately provide excellent continence.

Consent should include residual SUI, persistent or new urgency, retention, infection, erosion, mechanical failure and future revision. Explain the consequence of explantation and how future catheterisation/endoscopy will require precautions.

Evidence meaning

AUSCO26: prospective single-arm AMS 800 study, 115 implants; 61/101 evaluable participants reported zero pads (60.4%). Different endpoints had different denominators. **CCSAFETY**: uncontrolled early ContiClassic report, 116 recipients, 93.2% twelve-month Kaplan-Meier survival. Full detailed continence outcomes were not established from the accessible abstract. **AUA24** supports lifetime counselling; **RT22** adds observational radiation-associated risk. These data cannot rank devices against one another.

A speaking line

"These percentages answer different questions. I can counsel meaningful improvement and likely future maintenance without promising permanent dryness."

Be ready for

Kaplan-Meier estimates account for differing follow-up and censoring; 93.2% is not simply a pad-free proportion of 116 patients.

SLIDE 11 TEACHING 10

Case 1: six pads and urgency

Clinical explanation

This fictional patient has demonstrated SUI plus urgency leakage and DO, with reassuring compliance and emptying. Separate what the cuff can treat from what the bladder may continue to do. His walking goal may improve substantially even if urgency needs ongoing treatment.

For general AUS practice, DO alone is not an automatic exclusion. The decision depends on symptom dominance, storage pressures, treatment burden and expectations. In this ContiClassic discussion, optimise bothersome storage symptoms and establish control consistent with the actual label before implantation. Reassess if the symptoms or safety picture change.

Evidence meaning

DO²⁴ reviewed heterogeneous AUS/sling studies and did not establish a statistically significant overall excess failure sufficient to deny otherwise appropriate surgery solely for DO. OAB¹¹ found OAB can persist despite improved continence. DOFAIL²³ associated DO with earlier reintervention among a selected group already undergoing reintervention; it does not provide an all-recipient absolute failure probability. CCIFU requires associated urge/overflow/instability to be treated and controlled or resolved and contraindicates irresolvable instability.

A speaking line

"I would optimise the bothersome urgency, confirm safe storage and reassess. DO itself is not a blanket exclusion, but the selected device's instructions remain relevant."

Be ready for

Do not promise that stopping stress leakage will cure urgency. This case has no single mandatory answer for every AUS/device context.

SLIDE 12 TEACHING 11

Urgency treatment and emptying

Clinical explanation

Address fluid/caffeine patterns, constipation, infection when suspected and contributing medication. Choose bladder training and medication with comorbidity, cognition, interactions and residual urine in mind. An antimuscarinic may add cognitive burden and affect emptying; beta-3 agonists have their own contraindications and monitoring needs.

Before botulinum toxin, discuss possible retention, catheter competence and safe repeated cystoscopic access through a future cuff. Improving storage can create an emptying problem. Neuromodulation is another appropriate OAB-pathway option, depending on the patient. After AUS, reassess persistent urgency rather than automatically blaming the implant or simply increasing medication.

Evidence meaning

AUAOAB24 supplies evidence-based OAB treatment choices and shared decision-making. BAUS25 situates mixed symptoms within the continence pathway. OAB11 shows that storage symptoms often persist despite improved SUI. CCIFU supplies device-specific bladder precautions. None establishes one universally superior sequence of medication, botulinum toxin and AUS, or a single AUS-specific prescription.

A speaking line

"Stress leakage and urgency need separate treatment goals. Before escalating bladder treatment, I want a credible emptying and future-access plan."

Be ready for

Retention risk is especially relevant when the patient can operate a pump but cannot perform the drainage strategy that might become necessary.

Case 2: weak contraction and residual urine

Clinical explanation

Repeated raised residuals plus slow voiding and abdominal straining deserve explanation. Check for narrowing, medication effects, neurological disease and laboratory inhibition. A low-flow void with weak Pdet may indicate DU, but one pressure-flow episode must be interpreted beside usual emptying.

The AUS temporarily opens for voiding, which makes its physiology different from a permanently fixed compressive outlet. Nevertheless, a man who relies on abdominal straining may not empty equally well after implantation. My initial answer is investigate, establish a reliable emptying/rescue plan, then reconsider candidacy. Unresolved retention and inability to manage drainage favour deferral.

Evidence meaning

DU23 retrospectively assessed 78 patients, including 55 with BCI below 100; postoperative residuals did not significantly differ by DU status. It excluded complications requiring revision within three months, weakening safety inference. AUA24 and ICSUDS17 support appropriate diagnosis and test quality. CCIFU warns about irreversible obstruction and uncontrolled overflow. A research BCI threshold is not a validated refusal or acceptance criterion.

A speaking line

"I would investigate before deciding. The issue is whether he can empty reliably with the cuff opened and manage a realistic rescue strategy."

Be ready for

No universal residual-volume cutoff is established here. Reassuring selected DU outcomes cannot be extrapolated to every acontractile bladder or severe chronic retention case.

SLIDE 14 TEACHING 13

Catheterisation with a cuff in place

Clinical explanation

Clarify why catheterisation is needed and how often. Recurrent narrowing is a different problem from occasional retention or chronic DU. Check actual competence before implantation, including access and recognising resistance. A plan may need a specialist alternative drainage route if repeated or prolonged urethral access is likely.

Before passing an instrument, fully deflate the cuff and deactivate the actual system. Use appropriate atraumatic instrumentation and stop with unexplained resistance. The nominal cuff size does not equal the free internal lumen: urethral tissue lies within it. A single French-size rule cannot replace individual/device assessment.

Evidence meaning

CIC23 described 57 selected patients with a CIC history; only 18 continued or started CIC after AUS. Non-significant differences do not demonstrate equal safety. CATH13 associated catheterisation lasting more than 48 hours with erosion in an observational cohort. That exposure definition is not a guarantee of safety up to 48 hours. CCIFU and ICS16 provide instrumentation precautions and device-aware drainage principles.

A speaking line

"The patient and any future clinician must know there is a cuff. Deflation and deactivation come before instrumentation, with the drainage route planned in advance."

Be ready for

Careful CIC and prolonged indwelling catheterisation are different exposures; do not attribute identical risk to them.

Detrusor overactivity and poor compliance

Clinical explanation

DO is an involuntary contraction during filling. Poor compliance is a sustained pressure rise for an increase in volume. The sketch shows a phasic DO example; DO can also be terminal and sustained, so waveform shape alone is insufficient. Interpret the event, subtraction and filling history.

Closing an incompetent outlet can permit more urine storage. If that storage occurs at high detrusor pressure, pad use may improve while bladder/upper-tract safety remains concerning. Check renal function and upper tracts where indicated, optimise the bladder and reassess. Refractory unsafe storage may require reconstructive or diversion discussion before continence surgery.

Evidence meaning

UDS09 found adverse preoperative UDS features did not necessarily worsen postoperative pad use, while specifically raising concern that good continence does not establish safe storage. ICS16 and BAUS25 support bladder assessment. CCIFU warns about poor compliance/small fibrotic bladder and requires relevant bladder conditions to be addressed. The diagram has arbitrary units and supplies no threshold.

A speaking line

"A dry result is insufficient if the bladder stores at unsafe pressure. I separate DO from compliance and assess the safety question directly."

Be ready for

Avoid importing a neuro-urology pressure threshold as a universal non-neurogenic AUS eligibility cutoff. Interpret the actual patient and study quality.

SLIDE 16 TEACHING 15

Three different sites of narrowing

Clinical explanation

Name the anatomy precisely. An anterior stricture involves urethra/spongiosum. Bladder-neck contracture typically follows prostate surgery with tissue remaining. After radical prostatectomy, narrowing at the bladder-to-urethra join is VUAS. Different labels imply different imaging, access, reconstructive operations and risk to continence.

Review prior incisions, dilations and repairs, including how quickly the lumen narrowed again. A single successful passage of a scope does not establish stability. Look for a convincing interval without recurrence supported by symptoms, flow, residual and appropriate endoscopic assessment. Plan future tumour surveillance or treatment of radiation bleeding before a cuff complicates access.

Evidence meaning

EAUSTR26 supplies site-specific narrowing/reconstruction pathways. BAUS25 and AUA24 require relevant outlet pathology to be assessed and treated before continence surgery. Combining anatomical patency with clinical durability is practical synthesis. The teaching image illustrates general relationships and cannot define a patient's stricture length or reconstructive plane.

A speaking line

"I want the exact site, prior treatment and recurrence pattern, because a temporarily open lumen may still be an unstable outlet."

Be ready for

BNC and VUAS should not be used interchangeably in a referral or operative discussion.

Reassessment and urethral stability

Clinical explanation

The intervals refer to different pathways. AUA discusses cystoscopy at least four to six weeks after symptomatic BNC/VUAS treatment to document improvement/stability. EAU's three-to-six-month advice is tied to redo vesicourethral anastomosis. BAUS recommends at least six months without recurrence after treatment of post-prostatectomy narrowing.

These are opportunities to establish a durable outlet, not calendar guarantees. Frequent restenosis, irradiated tissue and continuing endoscopic treatment can remain decisive after an interval has elapsed. Staging also avoids repeated treatment through a newly cuffed urethra.

Evidence meaning

AUA24, EAUSTR26 and BAUS25 provide the actual source-specific intervals; ICS16 gives broader consensus context. They rely largely on consensus and observational experience. No randomised timing trial establishes one uniquely safe waiting period for all repairs. AUA's earlier reassessment must not be quoted as approval for immediate implantation in every reconstructed urethra.

A speaking line

"These numbers describe different clinical situations. I need evidence of stable patency and a realistic future-access plan, alongside the relevant guideline interval."

Be ready for

If challenged about a shorter interval, explain the specific reconstruction and recurrence history before defending a number.

Case 3: radiation and recurrent VUAS

Clinical explanation

This fictional patient has severe SUI when the outlet is open, but patency depends on repeated endoscopic intervention after prostatectomy and salvage radiation. Implanting now would place a cuff within an ongoing access/restenosis problem. Assess storage function and radiation cystitis as well as the lumen.

My preferred initial answer is stage a specialist reconstructive plan. Stable reconstruction followed by AUS may become appropriate. In a devastated outlet with unsafe or intolerable bladder function, a different drainage/diversion endpoint may offer better quality of life. The patient's wish for continence deserves a durable plan rather than another isolated procedure.

Evidence meaning

RT22 pooled 18 observational studies and associated radiation with worse complete continence and more erosion/explantation. These pooled associations do not give his individual probability or prove that radiation alone contraindicates AUS. [BAUS25](#), [EAUSTR26](#) and [AUA24](#) supply outlet stability and reconstructive guidance. The proposed sequence applies those principles to a fictional complex case.

A speaking line

"I would defer the implant while we agree a durable reconstructive endpoint. Radiation adds risk, but the immediate problem is recurrent stenosis and likely further access."

Be ready for

Do not convert an odds ratio into an absolute patient risk. Explain what extra information would allow reconsideration: durable patency, acceptable bladder function and a manageable access plan.

SLIDE 19 TEACHING 18

AUS after urethral reconstruction

Clinical explanation

Prior urethroplasty is not a single exposure. Identify the repair site, whether it transected the urethra, grafts, posterior reconstruction, fistula history and previous cuff erosion. Retrieve operative reports to understand remaining blood supply and viable cuff-bearing tissue.

Limit mobilisation and preserve remaining attachments rather than reproducing virgin-urethra dissection. A convenient circumference does not prove healthy tissue. An unsafe plane can justify an alternative approach, staging or seeking reconstructive expertise. Earlier reconstructive choices and later continence surgery are connected decisions.

Evidence meaning

URETH26 retrospectively examined 178 cases from 15 institutions; complications requiring explantation occurred in 56.2% after transecting versus 23.5% after non-transecting urethroplasty. Case selection, anatomy, disease severity and follow-up can confound that comparison. BAUS25, AUA24 and ICS16 provide the clinical framework. The cohort supports detailed counselling/history, but does not prove every transection caused failure or was avoidable.

A speaking line

"I need the reconstruction history in detail because it changes the tissue available for the cuff and the safety of the next dissection."

Be ready for

The citation's 2026 issue year is genuine; it appeared online in September 2025. Avoid claiming randomised superiority of non-transecting repair.

Transcorporal placement: role and costs

Clinical explanation

Transcorporal placement can incorporate tunical tissue under the cuff and avoid a hazardous dorsal urethral plane. Its rationale should address a particular anatomical problem. It also involves corporal dissection, bleeding risk, erectile consequences and potential implications for penile prosthesis surgery.

Comparisons are difficult because surgeons often reserve this approach for the most compromised urethras. Worse outcomes can reflect case severity; apparently favourable outcomes can reflect different selection or follow-up. Tandem cuffs and other salvage strategies also add complexity or consume tissue, and need an explicit indication.

Evidence meaning

TC20's 76-patient fragile-urethra retrospective series favoured transcorporal revision outcomes; its erosion difference was not statistically significant. TC23's different high-risk series associated transcorporal placement with more infection/erosion-related explants. BAUS25 and AP23 recognise specialist salvage choices. Conflicting observational studies do not establish universal protection from erosion or prove that transcorporal placement itself causes the worse result.

A speaking line

"I would choose transcorporal placement for a defined dissection problem, while explaining its costs and the uncertainty about comparative erosion protection."

Be ready for

Confounding by indication means the treatment was selected because the patient was already at higher risk. It is a major limitation of these series.

SLIDE 21 TEACHING 20

The fifteen-minute wash question

Clinical explanation

Ask which activity someone means by a wash: patient preoperative hygiene, operative skin preparation, surgical hand antisepsis, wound irrigation, external device dipping or internal device filling. They have different objectives and instructions. A timing recommendation for one cannot be transferred to another.

The sources do not establish a fixed fifteen-minute preparation as an AUS outcome standard. BAUS does recommend a five-minute topical field scrub, but explicitly bases it on Delphi consensus and extrapolation. Correct product, anatomical suitability, complete coverage and drying remain essential; simply extending elapsed time does not correct those deficiencies.

Evidence meaning

BAUS²⁵ provides the actual consensus recommendation. NICE supports preoperative soap washing and appropriate antiseptic site preparation. WHO⁰⁹ concerns hand hygiene, including surgical hand preparation. PREP²⁴ compares different povidone protocols across sequential eras, rather than randomising fifteen minutes against a shorter duration. The slide challenges an unproven fixed ritual while retaining proper antisepsis.

A speaking line

"Before discussing duration, I would identify the intervention and product. A five-minute BAUS field-scrub consensus and hand-preparation instructions answer different questions."

Be ready for

Absence of a direct duration trial does not mean antisepsis is optional, or that a manufacturer's required application/drying instructions can be shortened.

Skin culture reduction and clinical infection

Clinical explanation

A culture-positive skin swab is a surrogate outcome. Reducing skin flora is biologically plausible protection, but clinical implant infection also depends on patient factors, handling, contamination and subsequent events. The graph compares immediate post-preparation culture positivity, not AUS infection rates.

Yeung's randomised comparison found 8% culture positivity with chlorhexidine-alcohol versus 32% with povidone-iodine. That is a 24-percentage-point absolute reduction in this endpoint. It is not a 24-point reduction in implant infection, nor a fifteen-minute duration trial. General SSI guidance and exact product instructions remain necessary around genital skin/mucosa and alcohol pooling.

Evidence meaning

YEUNG13 enrolled 100 initial genitourinary prosthesis procedures, 50 per arm, and primarily measured cultures. PREP24's 156 AMS 800 procedures did not show a clear benefit from intensified preparation; inconsistent reported group counts/rates and sequential-era confounding limit interpretation. NICE supplies general site preparation/hair-removal guidance. CHLORA26 supplies restrictions and drying/flammability advice for a particular product. BAUS25 extrapolates implant-preparation recommendations.

A speaking line

"This is good evidence that one preparation reduced immediate skin culture positivity in that study. The clinical AUS-infection and duration questions remain less certain."

Be ready for

Check allergy, permitted sites and drying. Do not give an infection-prevention NNT derived from a culture endpoint.

SLIDE 23 TEACHING 22

Urine and antibiotic disagreements

Clinical explanation

Treat symptomatic infection and reassess elective implantation. Asymptomatic bacteriuria is bacteria in urine without attributable UTI symptoms, and is a different question. Routine AUS implantation normally does not enter the urinary mucosa, apart from catheterisation; a concurrent mucosal-trauma procedure may require a different pathway.

The disagreement is real. BAUS asks for preoperative urine assessment/treatment of infection; IDSA specifically suggests against screening or treating ASB solely for AUS implantation, with standard perioperative prophylaxis. Choose prophylaxis through local implant/microbiology policy, previous isolates, allergies and renal function. Give it at the correct pre-incision time and redose where indicated.

Evidence meaning

IDSA19's device recommendation is weak with very-low-quality evidence. BAUS25 supplies UK perioperative guidance. AUAABX20 supports appropriate timing, redosing and avoiding unnecessary prolonged prophylaxis. ABX18's claims-based cohort found no association between postoperative oral antibiotics and lower early AUS explantation. CCIFU's retrieved version describes 48 hours IV and five days oral antibiotics after discharge. That label description is not proof of benefit, and should be reconciled with applicable regional instructions and local experts.

A speaking line

"I would separate symptomatic infection from ASB, then document the agreed implant-service and microbiology protocol, including any device-label difference."

Be ready for

Do not silently present one universal antibiotic regimen or imply postoperative antibiotics have proved useless in every subgroup.

Contamination control during implantation

Clinical explanation

Prepare the sterile handling area before opening the implant. Limit unnecessary hand-offs and contact with skin, and use a deliberate clean handling pause. Fresh sterile gloves before device handling and after contamination concern are reasonable practical controls.

Keep wound irrigation, external dipping and internal filling separate. An absorbent hydrophilic coating does not establish clinical benefit from any chosen antibiotic mixture. A remembered AMS 800 recipe may be incompatible with another system. Use the actual device instructions and its operative protocol for preparation.

Evidence meaning

TOUCH23 compares a minimal-touch approach with a historical cohort. Lower crude infection figures are encouraging, but twelve-month cumulative-incidence differences did not meet statistical significance, and the bundle cannot identify the benefit of a single component. NICE does not recommend wound irrigation solely to prevent SSI. AP23 supplies AMS 800 consensus, including regional antibiotic context; CCIFU supplies ContiClassic-specific instructions/compatibility boundaries.

A speaking line

"Contamination control has a strong practical rationale. The bundle studies cannot tell us which individual step accounts for any improvement."

Be ready for

Wound irrigation for another specific surgical purpose is a separate question. The cited SSI recommendation does not settle all uses of irrigation or device filling.

The pre-implant theatre checklist

Clinical explanation

The checklist turns planning into a team decision before the implant is opened. Confirm indication, tissue concerns, infection assessment, prophylaxis and redosing. Check the antiseptic is suitable for the field and has dried. Confirm model, compatible components, cuff availability, pressure specification and preparation instructions.

Agree drainage and the response to urethral injury or contamination. These are meaningful stop conditions. A checklist is most useful when the scrub practitioner, surgeon and anaesthetic team understand the plan and can identify a missing condition before implantation.

Evidence meaning

NICE supplies general infection-prevention standards; BAUS25 supplies the AUS pathway; AUAABX20 supports antimicrobial principles; CCIFU supplies product requirements. The rows deliberately distinguish guidance-supported actions from local/product choices and expert implementation. There is no cited validation that this exact checklist predicts an outcome, and a fixed wash time is not a proxy for overall quality.

A speaking line

"Before opening the implant, the team should know the tissue plan, the product plan, the drainage plan and the findings that would change the endpoint."

Be ready for

The checklist supports reproducibility; it does not establish the superiority of a particular irrigant or duration of postoperative prophylaxis.

SLIDE 26 TEACHING 25

Positioning and exposure

Clinical explanation

Positioning must permit safe perineal access while respecting hip mobility and protecting pressure points. Plan abdominal/balloon and scrotal/pump access at the same time. Previous surgery, mesh, abdominal scars, reconstruction and an existing penile prosthesis can change the route.

Poor exposure should prompt deliberate improvement of positioning, incision or retraction. Excessive traction to work through a limited window transfers force to the urethra and distorts tissue measurement. The aim is controlled access to the actual plane, not a cosmetically small incision at any cost.

Evidence meaning

AUA24 supports a single-cuff perineal approach for standard AUS implantation. ICS16 and AP23 supply operative consensus and anatomical planning principles centred on AMS 800. The positioning details are expert surgical implementation, not comparative trials proving each manoeuvre reduces erosion. The UroOps image is a general schematic and cannot establish this patient's scar planes.

A speaking line

"I would plan all component access before dissection, and improve exposure rather than compensating for it with traction."

Be ready for

Alternative access may suit selected anatomy and experienced operators. State why it solves a defined problem, rather than implying convenience alone determines the approach.

SLIDE 27 TEACHING 26

Bulbar dissection and tissue preservation

Clinical explanation

The cuff needs viable urethra and supporting spongiosum. Circumferential access does not require extensive skeletonisation. Prior reconstruction, erosion or radiation can change which segment and plane are safe. Limit mobilisation to what is needed and preserve remaining vascular attachments.

The dorsal relationship to the corporal bodies deserves controlled dissection and an adequate view. Blind passage or forceful spreading through scar can injure the urethra. If the plane disappears, improve the view, reconsider the cuff site, obtain specialist help or stage the operation. Avoid indiscriminate thermal injury while obtaining haemostasis.

Evidence meaning

ICS16 and AP23 supply surgical consensus. CCIFU warns against cuff implantation where spongiosal support is inadequate. URETH26 gives observational evidence that reconstruction history influences later AUS outcomes. Tissue preservation has a clear physiological rationale; the independent effect of every dissection manoeuvre on erosion has not been quantified by high-quality comparative trials.

A speaking line

"We need enough controlled exposure for the cuff while preserving the tissue and attachments that support the urethra."

Be ready for

The image illustrates the relationship; it cannot demonstrate perfusion, prove the safety of a plane or substitute for the operative finding.

Cuff measurement and fit

Clinical explanation

Measure the tissue the cuff will actually surround. Retraction stretches the urethra; a tight measuring tape compresses it. Included scar, fat or muscle, oedema and haematoma can enlarge or distort the apparent circumference. Release traction, confirm the enclosed tissue and use the system's supplied method.

A small cuff can produce excessive compression, retention or injury; a large cuff may leave inadequate coaptation. Higher pressure cannot reliably rescue a sizing error. In transcorporal or reconstructed anatomy, the enclosed tissue differs, so an unsupported numerical correction is inappropriate. Recheck a reading that does not fit the operative appearance.

Evidence meaning

ICS16 and AP23 provide AMS 800 consensus sizing principles; CCIFU governs the actual ContiClassic system. CCSAFETY reported four revisions attributed to mistaken sizing among its eight revisions. That illustrates a clinically important error, but does not identify a superior sizing method or prove that more cuff-size choices improve survival.

A speaking line

"A good measurement comes from undistorted tissue and the correct product method. A stronger balloon does not compensate for a poor fit."

Be ready for

Cuff circumference in centimetres is not a catheter lumen size. Distinguish size selection from pressure selection.

SLIDE 29 TEACHING 28

Urethral injury or contamination

Clinical explanation

If urethral integrity is uncertain, assess it with appropriate controlled testing or endoscopy. Once injury is recognised, the endpoint changes to repair and safe drainage rather than completing an elective implant through compromised tissue. The repair and timing of later continence surgery require patient-specific specialist judgement.

If sterility is lost, identify what became contaminated, replace affected single-use items and restore the clean setup. Do not assume an improvised antiseptic bath restores a contaminated implant to its original sterile condition. It may also damage materials or breach product instructions.

Evidence meaning

BAUS25 supports stopping implantation with recognised urethral injury. ICS16 and AP23 describe integrity assessment and troubleshooting principles. CCIFU provides single-use, sterility and handling precautions. These are consensus/label-based stop conditions, rather than randomised tests of proceeding through an injury versus abandoning the implant.

A speaking line

"A recognised injury changes what success means today: repair and safe drainage. Completing the planned implant is no longer the appropriate immediate endpoint."

Be ready for

Avoid giving an invented universal healing interval for reimplantation. It depends on injury, tissue, infection, repair and demonstrated stability.

Balloon pocket and pressure selection

Clinical explanation

The balloon is the hydraulic pressure-regulating component. Its nominal pressure range is a device specification, distinct from bladder detrusor pressure and cuff circumference. Selecting the component and safely creating a pocket are separate decisions.

Pelvic scars, mesh and altered retropubic anatomy may change balloon access. Alternative pockets need a deliberate anatomical plan and appropriate expertise. A higher nominal range increases potential compression; it cannot make an unsuitable cuff site, poor fit or failing urethra healthy.

Evidence meaning

CCIFU lists ContiClassic's 60-69 cmH₂O component among several available ranges; the slide does not imply it is the only range or a universal selection. [AP23](#) describes common AMS 800 use of 61-70 cmH₂O; [ICS16](#) supplies general AMS 800 operative context. Neither authorises transferring AMS filling volumes, preparation or pressure-selection rules into ContiClassic. The separate current OR protocol was not obtained.

A speaking line

"These labels belong to different systems. The pocket, pressure component and fill protocol each need their own correct decision."

Be ready for

60-69 and 61-70 cmH₂O are not eligibility thresholds or target storage pressures. Do not prescribe an unverified fill volume.

SLIDE 31 TEACHING 30

Pump position and patient access

Clinical explanation

Use the accessible side agreed in clinic. The pump needs a palpable, functional position without crowding the testis, threatening skin or excessive tension. A pocket that is too large can permit migration; one that is too tight can cause discomfort and impaired access.

Before closure, consider position when standing and seated, orientation of the controls and tubing traction. At activation, the patient should demonstrate full operation rather than only locate the pump. A useful position on an anatomical model does not establish easy access in an oedematous postoperative scrotum.

Evidence meaning

AP23 and ICS16 describe consensus placement principles. CCIFU defines controls and product-specific operation. The accessible-side assessment and rechecking at activation are practical implementations of functional eligibility. These sources do not establish a universal optimal pocket dimension or side, and the schematic does not replace an individual access assessment.

A speaking line

"Pump position is part of the continence result: the patient must reach, identify and operate it comfortably after healing."

Be ready for

Dominant hand alone does not choose the side. Reach, anatomy, disability and another implanted scrotal device may matter more.

SLIDE 32 TEACHING 31

System checks and handover

Clinical explanation

Trace every tube for tension, kinks, sharp turns or entrapment under closure. Confirm compatible components and secure connections, and prepare/purge the system using its product protocol. Check function, then confirm the cuff is open and the device correctly deactivated. A presumed button press is insufficient verification.

Handover records the device model, cuff site/size, pressure component and balloon position, deactivated state, drainage plan and contact route. Future clinicians need to know about the cuff before catheterisation. Early removal after uncomplicated implantation should be paired with verified emptying and a safe retention response.

Evidence meaning

CCIFU supplies the product requirements; [ICS16](#) and [AP23](#) give AMS 800 system-check and aftercare consensus. [CATH13](#) adds observational evidence of catheter-associated erosion. Brief catheterisation recommendations from AMS consensus are not independently verified ContiClassic duration rules. Detailed connector, filling and purging sequences remain dependent on the actual operative protocol.

A speaking line

"I want verified device function and an open, deactivated cuff, followed by a handover that protects the patient from unsafe future instrumentation."

Be ready for

Drainage after injury, reconstruction or retention may differ from uncomplicated implantation. Do not quote one universal catheter size or duration.

SLIDE 33 TEACHING 32

UroOps 3D: anatomy and component routes

Clinical explanation

Use the four-minute demonstration to connect anatomy to decisions. Start with viable cuff-bearing bulbar tissue and the dorsal plane. Move to pump accessibility, then the balloon pocket and finally tubing without tension or sharp turns. Explain that prior surgery changes these spaces.

The hydraulic cycle can reinforce why pump operation opens the cuff temporarily for voiding. Keep generic mechanism separate from exact activation/deactivation instructions. If the demonstration fails, describe the same component relationships verbally and return to the talk; the supplied notes provide a fallback.

Evidence meaning

ICS16 and AP23 support general anatomical/surgical teaching centred on AMS 800; CCIFU governs ContiClassic operation and placement boundaries. The UroOps model is a teaching schematic with clinical review pending in the deck. A convincing image cannot validate perfusion, scar planes, dimensions or a safe real-life trajectory. No new clinical study is cited by the model itself.

A speaking line

"I will use the model to explain why these routes matter: tissue for the cuff, access for the pump, safe balloon placement and an unstrained tubing route."

Be ready for

The illustration of relationships should not become a device preparation tutorial or an assertion of validated surgical anatomy. Rehearse the live page independently of this superseded draft.

Aftercare, activation and urgent review

Clinical explanation

After uncomplicated surgery, remove the catheter early where appropriate and verify emptying. Retention needs assessment of cuff state, oedema, fit and outlet pathology, with safe drainage. Repeated blind attempts can turn a functional issue into urethral injury.

Activation is a review and teaching visit: assess healing, operate the device with the patient and check emptying. New/worsening pain, erythema, discharge, fever or difficulty voiding warrants earlier specialist review. Established infection or erosion usually needs explantation, rather than assuming antibiotics alone can preserve the system.

Evidence meaning

CCIFU describes four to six weeks deactivated and deferring activation until pain/oedema settle. BAUS25 and AUA24 supply follow-up, patient education and complication-management principles. CATH13 reinforces catheter precautions. These do not make the activation calendar override symptoms or establish a universal drainage plan after every operation.

A speaking line

"At activation I need a healed, usable device and a patient who can demonstrate operation and empty safely. Red flags are assessed before routine follow-up."

Be ready for

Give a device card and clear urgent contact route. Knowledge of the implant must reach the clinician holding the catheter or scope.

SLIDE 35 TEACHING 34

Case 4: leakage after three good years

Clinical explanation

Distinguish true recurrence after initial benefit from leakage that never resolved. Ask whether symptoms are stress-related, urgency-related or overflow, and observe pump use. Check device state, urine and residual; examine components and arrange appropriately deactivated cystoscopy for erosion/urethral assessment. Investigate mechanics/fluid loss and bladder dysfunction as indicated.

Avoid booking a smaller cuff solely because time has passed. The diagnosis may be retraining, bladder treatment, a defective component, altered coaptation, erosion or another outlet problem. Revision may involve selected component replacement or the whole system depending on findings and system age.

Evidence meaning

ATROPHY20's selected revision cohort attributed relatively few failures to atrophy and more to pressure-regulating balloon failure. Its erosion exclusions, surgeon setting and definitions prevent universal prevalence claims. BAUS25 and AUA24 support mechanism-based evaluation; CCIFU describes device-related complications and operation. None justifies automatic cuff downsizing or pressure escalation at three years.

A speaking line

"I would investigate use, urethra, mechanics and bladder before choosing revision. Elapsed time does not diagnose urethral atrophy."

Be ready for

Shrinking the cuff around an already compromised urethra may consume tissue without solving the true cause of leakage.

Established benefit and unresolved questions

Clinical explanation

AUS offers substantial improvement for appropriately selected men, with future revision part of the treatment burden. Tissue preservation, contamination control and planned access have strong rationale. The strength of that rationale differs from evidence that a particular salvage manoeuvre or irrigant improves outcomes in a comparative trial.

State uncertainty where it changes counselling or planning. There is no single proven salvage strategy for every fragile urethra, no direct universal preparation duration and no matched long-term ContiClassic efficacy comparison among the cited data. This helps an expert audience discuss research questions without weakening established patient-selection principles.

Evidence meaning

MASTER22 provides randomised comparative information in selected primary cases. TC20 and TC23 show conflicting salvage associations in different retrospective populations. PREP24 offers limited preparation data with reporting concerns. CCSAFETY supplies early uncontrolled ContiClassic safety/survival information. Combining those sources does not produce stronger certainty than their designs allow.

A speaking line

"We have established benefit in selected patients, useful operative principles, and several comparative questions that remain unanswered."

Be ready for

An absence of comparative proof is not proof that two options are equivalent. Explain the relevant uncertainty rather than calling everything low-quality evidence.

SLIDE 37 TEACHING 36

Five decisions before implantation

Clinical explanation

Summarise the preoperative pathway: establish SUI and the desired improvement; demonstrate pump function/access; investigate anatomy, storage and emptying; distinguish DO, DU and poor compliance; and establish a stable urethra with a future-access plan.

These steps interact. Botulinum toxin may improve urgency while creating a catheter requirement. Treating a stenosis may reveal more severe SUI. A pump demonstration may identify a functional limitation despite a strong anatomical indication. Return to the patient's goal after each major finding.

Evidence meaning

BAUS25, EAU26 and AUA24 provide the assessment pathway with differing UDS positions. ICS16 supports functional/device assessment. DU23 supplies limited selected-cohort evidence that a weak contraction does not automatically exclude AUS. This is a practical recap, not a validated score or five new independent eligibility thresholds.

A speaking line

"Before theatre I want a demonstrated mechanism, a usable pump, credible storage and emptying assessment, and a durable outlet with future access considered."

Be ready for

You can apply these principles to the earlier cases: optimise case 1, investigate case 2 and obtain a staged reconstructive plan for case 3.

SLIDE 38 TEACHING 37

Five decisions protecting the result

Clinical explanation

Complete the ten messages with appropriate preparation/prophylaxis, preserved tissue and valid sizing, actual device instructions/stop conditions, verified deactivation/drainage/patient teaching, and a diagnosis before revision.

The common theme is correcting uncertainty before it creates a difficult complication. Improvised implant preparation, forcing an unsafe plane, escalating pressure to compensate for poor fit, or downsizing without diagnosing recurrence all convert an unresolved question into tissue or device risk.

Evidence meaning

CCIFU supplies product-specific requirements. NICE supplies infection-prevention principles. AP23 supplies AMS 800 operative consensus; ATROPHY20 cautions against assuming all recurrence is atrophy. BAUS25 provides the UK pathway. These are integrated take-home decisions from earlier slides, not a newly tested prevention bundle with a quantified risk reduction.

A speaking line

"Protect the tissue, follow the exact system's instructions, verify function and drainage, and diagnose recurrent leakage before selecting the next operation."

Be ready for

The goal is useful continence over time with safe storage and emptying. An initially dry result alone does not measure the whole pathway.

SLIDE 39 TEACHING 38

Discussion

Clinical explanation

Use the five-minute discussion to expose reasoning. Ask which finding changes a colleague's choice in the cases, then ask what authority supports that decision: guideline, IFU, clinical study or local practice. Keep answers focused enough to allow a contrasting view.

For bladder dysfunction, unsafe storage and unresolved emptying deserve different responses from DO alone. For outlet stability, distinguish repair type, recurrence history and future access. For preparation/salvage, specify a population and a meaningful endpoint before suggesting a trial.

Evidence meaning

This slide introduces no new scientific result. Sources depend on the question and are decoded in slides 43-46. A show of hands is audience opinion; it does not become a formal Delphi consensus or clinical evidence.

A speaking line

"Tell us what you would do and the specific finding that would make you change the plan. Is the reason a guideline requirement, a device instruction or your interpretation of the evidence?"

Be ready for

For a question beyond the retrieved evidence, state the uncertainty precisely. Avoid inventing a cutoff, protocol, local outcome or personal operative experience.

SLIDE 40 BACKUP / REFERENCES

Outcome evidence: device and endpoint

Clinical explanation

This backup table separates effectiveness, durability and early safety. AUSCO measures improvement and pad use twelve months after activation. Linder measures freedom from any secondary surgery over years. Wilson reports early ContiClassic revision/device survival in mixed indications. They answer different consent questions.

Read the denominator aloud. AUSCO implanted 115 men, but 97 supplied the pad-weight endpoint and 101 the pad-use endpoint. Linder's Kaplan-Meier estimates use follow-up and censoring; they are not proportions of a cohort all observed for ten years. ContiClassic's survival estimate is not standardised continence.

Evidence meaning

AUSCO26: 91/97 achieved at least 50% pad-weight reduction (93.8%); 61/101 reported zero pads (60.4%). **LINDER15:** 1,082 primary implants; freedom from secondary surgery 74% at five years and 57% at ten years. **CCSAFETY:** 116 implants, eight revisions and 93.2% twelve-month Kaplan-Meier survival. Short follow-up, missing data, historical practice and incomparable indications limit cross-study inference.

A speaking line

"I use these studies for different parts of consent: likely improvement, long-term maintenance and early experience with a newer system. They do not constitute a device comparison."

Be ready for

AUSCO is prospective but uncontrolled and manufacturer-funded. The ContiClassic abstract states no industry funding but describes industry-linked data collection and disclosed relationships; keep those distinctions accurate.

Comparative and preparation evidence

Clinical explanation

The strongest design here is MASTER's randomisation, but its result still needs its primary endpoint and margin. The sling-minus-AUS success difference was -3.4 percentage points, with a 95% CI from -11.7 to +4.8; the interval excluded the prespecified -15-point non-inferiority margin. It permits uncertainty about the actual difference and does not establish identical outcomes.

Yeung's randomisation addresses skin culture positivity; Bourgi's sequential-era comparison addresses intensified preparation and early device outcomes with substantial reporting limitations. Keep the endpoints separate.

Evidence meaning

MASTER²² enrolled 380 men and excluded prior sling/AUS and unresolved narrowing. Both groups improved, while several secondary results favoured AUS. YEUNG¹³ found 4/50 versus 16/50 positive post-preparation cultures (8% versus 32%). PREP²⁴ enrolled 156 AMS 800 procedures and reported no clear adjusted advantage; abstract and table group denominators differ. Its published infection percentages should not be used as dependable counselling estimates or clean inputs for an NNT.

A speaking line

"Randomisation strengthens a study's comparison, but it only supports the endpoint tested. A culture trial and a reporting-limited era comparison cannot settle AUS infection-prevention duration."

Be ready for

Non-significance is not equivalence. Small event counts and imprecise confidence intervals can leave clinically important differences unresolved.

Evidence gaps

Clinical explanation

Each gap needs a defined population, intervention and meaningful outcome. Routine versus selective UDS should measure management change, safety, patient outcomes and burden. DU studies should distinguish reliable emptying from unresolved retention, and report early complications rather than exclude them.

Reconstruction studies need repair type, radiation, recurrence and future-access details. Salvage comparisons need comparable baseline urethral risk. Preparation/antibiotic research should measure clinical infection or explantation and include adverse effects, rather than rely solely on skin cultures. New-device studies need standard continence definitions, revision indications, subgroup data and long follow-up.

Evidence meaning

The slide synthesises limitations across the cited literature and introduces no new trial result. These are unanswered comparative questions, not proof that the competing practices have equal outcomes or permission to bypass established precautions.

A speaking line

"The useful research question specifies which patients, which intervention and which outcome would change care. We should avoid using heterogeneous endpoints to claim certainty."

Be ready for

For late leakage, standardised investigation and failure definitions matter as much as comparing revision operations: the cause assigned to failure can depend on how thoroughly the device was investigated.

SLIDE 43 BACKUP / REFERENCES

References 1 of 4: clinical guidance and device use

This reference slide contains pathway, consensus, product and urodynamic-standard sources. These documents explain what a service recommends or a device requires. They should not be presented as nine independent outcome trials. The citation year usually means publication/edition year; some papers appeared online earlier.

BAUS25 | Bhatt and colleagues: BAUS PPI-SUI consensus, 2025

A UK consensus developed after a literature review and modified Delphi process. It provides the talk's assessment and perioperative pathway, including VUDS for surgical candidates, severity-dependent recovery periods and management of narrowing. It explicitly labels several recommendations as Delphi/extrapolation, including a five-minute field scrub. Consensus supports a consistent pathway where trials are incomplete; it does not prove every prescribed interval or technical step is uniquely optimal. Relevant full text was checked in the publisher-formatted article, with metadata/abstract cross-checked. [BAUS consensus](#).

EAU26 | EAU non-neurogenic male LUTS guidelines, 2026

The UI section recommends symptom assessment, diary/pad-weight measures and residual assessment. Its table weakly recommends invasive UDS when considering invasive treatment; its narrative acknowledges limited evidence about effects on management/outcome prediction. This is distinct from UDS advice before surgery for benign prostatic obstruction. It also supplies sling/AUS treatment context. A weak recommendation reflects uncertainty and the need to weigh clinical context; it does not make the test valueless. Relevant official online sections were checked. [EAU disease-management chapter](#).

AUA24 | AUA/GURS/SUFU incontinence after prostate treatment, amended 2024

This guideline permits surgery from six months for bothersome non-improving SUI despite conservative treatment, and recommends offering surgery at one year for persistent bothersome SUI. Preoperative cystourethroscopy is expert opinion; selective UDS to assist diagnosis/counselling is conditional Grade C. It supports functional pump suitability, counselling and evaluation of recurrence. Its BNC/VUAS discussion includes interval reassessment at least four to six weeks after treatment. Grade/strength varies by statement: a guideline citation does not make every recommendation high-certainty trial evidence. The official 41-page PDF was retrieved and relevant sections checked. [AUA guideline](#).

EAUSTR26 | EAU urethral-stricture guidelines

This source distinguishes anterior strictures from posterior stenosis and supplies reconstruction-specific pathways. After redo vesicourethral anastomosis, it discusses deferring continence surgery for at least three to six months to establish patency. That interval is not a general rule after every dilation or urethroplasty. Recurrent VUAS, radiation and bladder function influence reconstruction and the ultimate endpoint. Relevant official chapter sections were checked. [EAU male disease-management chapter](#).

ICS16 | Biardeau and colleagues: 2015 AUS consensus, published 2016

International consensus on AMS 800 assessment, operative principles, complications and aftercare. It supports sufficient dexterity/cognition, evaluation of bladder/outlet problems and careful device-aware instrumentation. It supplies much of the operative rationale in the deck, with recommendations based on varying evidence quality. Its older catheter and technical recommendations are system/context-specific; they should not become universal ContiClassic rules. Relevant full text was checked in the official ICS PDF. [ICS consensus](#).

AP23 | Chung and colleagues: Asia-Pacific AMS 800 consensus, 2023

Practical consensus on patient selection, investigation, operative troubleshooting and postoperative care. It discusses common AMS 800 pressure selection and component preparation, including regional issues such as coating availability and antimicrobial practice. Its proposed fill volumes or preparation sequence belong to AMS 800, not automatically to ContiClassic. It is a useful expert framework rather than a device-comparison trial. Relevant full text was checked. [Asia-Pacific consensus](#).

CCIFU | Rigicon ContiClassic instructions, REV.03, 2023

Manufacturer labelling: contraindications/precautions, components and device-related handling. It requires associated urge/overflow/instability to be treated and controlled or resolved; warns about poor compliance, thin spongiosum and irreversible obstruction; and requires cuff deflation/deactivation before instrumentation. It specifies four to six weeks deactivated and describes a longer postoperative antibiotic regimen than stewardship sources. These are product instructions, not proof of efficacy or of the benefit of prolonged antibiotics. The full public PDF was read. Country/lot applicability and the separate OR protocol remain unverified. [ContiClassic IFU](#).

ICSUDS17 | Rosier and colleagues: good urodynamic practice, 2017

Standards for uroflowmetry, cystometry and pressure-flow testing, including quality control, terminology, interpretation and reporting. They support checking that a trace is technically credible and representative before assigning DO, DU, compliance or obstruction. The title uses 2016; journal publication is 2017. This is a testing standard, not a trial showing one BCI or residual threshold determines AUS candidacy. Abstract/metadata were checked; this session did not independently review the full standard. [ICS urodynamic standard](#).

AUAOAB24 | Cameron and colleagues: AUA/SUFU idiopathic OAB guideline, 2024

Evidence-based options for evaluating and treating OAB through shared decision-making, including behavioural, pharmacological and procedural approaches. It helps explain medication choice, adverse effects, residual assessment and the retention implications of botulinum toxin. It addresses OAB generally; it does not establish an optimal sequence around AUS implantation. The publication abstract/metadata were checked, alongside the treatment principles already contained in the supplied deck. [OAB guideline publication](#).

SLIDE 44 BACKUP / REFERENCES

References 2 of 4: preparation, outcomes and storage symptoms

This page deliberately places general infection guidance beside product information and studies with different endpoints. In discussion, identify whether you are citing a recommended practice, a manufacturer's precaution, a surrogate endpoint or a clinical result. The studies below do not establish one common success rate.

NICE | NG125: surgical site infections

General surgical guidance on hygiene, hair removal, antisepsis, prophylaxis and wound care. Appropriate product selection and drying matter; if hair removal is necessary, clipping is recommended. The guideline advises against wound irrigation solely to reduce SSI. It does not test a fifteen-minute AUS preparation or authorise every antiseptic on genital mucosa. Relevant official recommendation text was checked. [NICE NG125](#).

WHO09 | WHO hand-hygiene guidance, 2009

A hand-hygiene standard, including surgical hand antisepsis. Its relevance is the distinction between preparing the operating team's hands/forearms and preparing the patient's skin. Product instructions govern application for the selected hand preparation. It is not an AUS-specific trial and cannot establish a required patient-field scrub duration. The official publication record was checked; no fixed AUS timing is inferred from it. [WHO guidance](#).

CHLORA26 | ChloroPrep Swabstick UK SmPC, accessed 2026

Product-specific information for chlorhexidine/alcohol swabsticks: permitted application, allergy, anatomical restrictions, drying and flammability. This presentation includes a 30-second application instruction with complete air drying; other presentations/procedures require their own instructions. The solution is for intact skin and excludes mucous membranes. The citation illustrates why the actual formulation matters, not a universal genital-preparation prescription. Relevant SmPC sections were checked. [ChloroPrep Swabstick SmPC](#).

IDSA19 | Nicolle and colleagues: asymptomatic bacteriuria guideline, 2019

Specifically suggests against screening for or treating ASB solely for AUS/penile-prosthesis implantation, while retaining standard perioperative prophylaxis. The recommendation is weak and based on very-low-quality evidence. Procedures causing urinary mucosal trauma have different recommendations. This helps separate a positive asymptomatic culture from symptomatic UTI and explains the disagreement with broader preimplant urine pathways. Relevant full guideline sections were checked. [IDSA ASB guidance](#).

AUAABX20 | Lightner and colleagues: antimicrobial prophylaxis statement, 2020

Consensus informed by a literature review on appropriate prophylaxis, timing, redosing and duration. Its general stewardship position favours the shortest necessary exposure, with single-dose prophylaxis for most urological procedures and no unnecessary continuation after case completion. It does not alone settle every high-risk implant or override applicable product precautions. The publication abstract was checked; reconcile local implant policy and current device instructions rather than issuing a universal prescription. [AUA prophylaxis statement](#).

CCSAFETY | Wilson and colleagues: first ContiClassic safety outcomes

Uncontrolled report of the first 116 implants, with mixed indications: eight revisions (6.9%), no reported infections and 93.2% twelve-month Kaplan-Meier survival. Revisions included fluid loss, sizing errors and dissatisfaction. Data collection included operating staff/company representatives; the abstract declares no industry funding and reports author relationships. Early survival is encouraging, but the design has no matched comparator and does not supply a verified standardised continence estimate. This is a journal paper, not a conference abstract; only abstract-level methods/results were independently accessible here. Online 2023; issue 2024. [Wilson study](#).

MASTER22 | Constable and colleagues: sling versus AUS RCT

UK multicentre non-inferiority trial, 380 randomised men (190 per group). Its primary continence definition used patient-reported ICIQ-UI questions at twelve months, with a 15-percentage-point margin. The intention-to-treat difference was -3.4 points (95% CI -11.7 to +4.8), meeting non-inferiority. Both groups improved; several secondary findings favoured AUS. Prior implants and unresolved narrowing were excluded. Non-inferiority on that endpoint does not mean identical efficacy across severity/radiation/revision subgroups. Trial abstract and NIHR report results were checked. [NIHR MASTER report](#).

UDS09 | Lai and colleagues: pre-AUS urodynamics, 2009

Selected cohort of 129 men with post-RP incontinence undergoing first AUS, mean follow-up 34.1 months. Several adverse preoperative UDS findings did not significantly worsen subsequent daily pad use. The authors specifically cautioned that a good continence result with poor compliance could still expose upper tracts to high storage pressure. Thus, this supports distinguishing pad outcomes from bladder/renal safety, not disregarding abnormal UDS. Abstract checked. [Lai UDS study](#).

OAB11 | Lai and Boone: preoperative OAB and mixed symptoms, 2011

Selected 129-patient first-AUS post-RP cohort. Preoperative OAB did not worsen overall continence results, but 71% with mixed SUI/OAB continued to have OAB and 23% with initially pure SUI developed new OAB. These are cohort-specific counselling observations, not universal probabilities. They explain why improvement in SUI and persistent urgency can coexist. Do not assume DO and OAB are interchangeable or borrow identical follow-up from UDS09. Abstract checked. [Lai OAB study](#).

SLIDE 45 BACKUP / REFERENCES

References 3 of 4: bladder function and the fragile urethra

Most studies on this reference slide are observational. They help identify concerns and selected feasible pathways, but do not establish an individual's absolute risk or prove which intervention prevents failure. Distinguish a whole implantation cohort from a cohort selected because patients already required reintervention.

DO24 | Bhatt and colleagues: preoperative DO systematic review, 2024

Eleven studies, 792 patients, combining post-prostatectomy AUS/sling surgery with heterogeneous definitions and reporting. It did not establish a statistically significant overall increase in failure sufficient to deny surgery solely for DO. Procedure subgroup differences are vulnerable to selection and heterogeneity; they are not a randomised AUS-versus-sling comparison in DO. It supports counselling about persistent storage symptoms and subsequent treatment. Abstract checked. [DO review](#).

DOFAIL23 | Krughoff and Peterson: earlier AUS reintervention, 2023

Single-centre analysis of 92 included men already undergoing AUS reintervention. DO was associated with earlier intervention (adjusted HR 1.95, 95% CI 1.22-3.1). This is a time-to-event association within a selected reintervention population. It cannot estimate the chance of failure among all primary recipients or demonstrate that treating DO prevents device failure. It tempers overly reassuring counselling without establishing a blanket contraindication. Abstract checked. [Earlier-failure study](#).

DU23 | Han and colleagues: AUS with DU, 2023

Retrospective 78-patient cohort, including 55 with BCI below 100. Postoperative PVR did not significantly differ between DU and non-DU groups, although flow remained lower in DU. Excluding complications requiring revision within three months can remove important early adverse outcomes. Selected DU can be compatible with AUS, but the study does not validate safe implantation in all unresolved retention or acontractile cases. BCI below 100 is its study definition, not a universal eligibility cutoff. Abstract checked. [Han DU study](#).

CIC23 | Krughoff and colleagues: intermittent catheterisation after bulbar AUS

Retrospective selected cohort of 57 men with CIC history; 18 continued/started CIC after AUS and 39 discontinued before implantation. The study found no statistically significant difference in erosion or later removal/replacement between groups. Small numbers and selection make a missed difference possible. It supports feasibility in some carefully planned patients, rather than establishing equivalence to men who never catheterise or a universal safe catheter size. Abstract checked. [CIC study](#).

CATH13 | Seideman and colleagues: prolonged catheterisation and erosion

Observational implantation/revision cohort of 200 included men. The exposure was urethral catheterisation longer than 48 hours; 17/44 exposed men had erosion. Catheterisation, radiation and revision remained associated with erosion on adjusted analysis. Confounding by why catheterisation was needed remains possible. The study supports careful specialist drainage planning and avoiding unnecessary prolonged urethral access; it does not show that 48 hours is a guaranteed safe boundary or that CIC carries identical risk. Abstract checked. [Catheterisation study](#).

UDS14 | Weissbart and colleagues: absent leak during intubated UDS

Of 169 men, 32 did not demonstrate incontinence with the urethral test catheter in place; all leaked after it was removed. The subgroup included frequent anastomotic narrowing and prior radiation, and had useful subsequent AUS outcomes. The main lesson is possible catheter masking of real stress leakage in a selected population. It does not justify accepting a diagnosis without appropriate clinical demonstration and anatomical assessment. Abstract checked. [Weissbart study](#).

RT22 | Zhang and Xu: radiation meta-analysis, 2022

Eighteen observational studies associated prior radiation with reduced odds of complete continence and greater erosion/explantation risk. For illustration, the pooled erosion OR was 2.00 (95% CI 1.15-3.45). An odds ratio is not a risk ratio or a personal probability; it requires an appropriate baseline risk to translate into absolute risk. Heterogeneous indications, revision mixes and follow-up limit precision. Radiation informs counselling and tissue planning; it is not automatically an exclusion. Abstract checked and used qualitatively. [Radiation meta-analysis](#).

URETH26 | Davis and colleagues: AUS after urethroplasty, 2026

Retrospective multicentre series, 178 cases from 15 institutions; median follow-up 33.5 months. Complications requiring explantation occurred in 56.2% after transecting versus 23.5% after non-transecting urethroplasty. This supports recording technique and counselling about compromised tissue, but cannot separate technique effects from stricture severity, anatomy, selection and follow-up. It does not establish that every transecting repair should have been avoided. Online September 2025; February 2026 issue. Abstract checked. [Davis cohort](#).

TC20 | Redmond and colleagues: transcorporal placement in fragile urethras

Retrospective series of 76 patients; 24 underwent transcorporal placement. Revision was 20.8% versus 36.5% ($P=.05$); erosion 8.3% versus 17.3% ($P=.09$), favouring transcorporal numerically. The erosion result was not statistically significant, and allocation was not randomised. This suggests a possible specialist role but cannot establish universal protection or equal functional outcomes from a non-significant comparison. Interpret alongside the different high-risk cohort in [TC23](#). Abstract checked. [Redmond study](#).

SLIDE 46 BACKUP / REFERENCES

References 4 of 4: salvage, preparation and durability

This final reference slide supports careful interpretation of salvage choices, recurrence diagnosis, infection-control practices and lifetime counselling. Early outcomes, clinical infection, pad reduction and freedom from secondary surgery remain different endpoints. Below, numerical findings always belong to the study population and time window described.

TC23 | Kurtzman and colleagues: high-risk transcorporal AUS, 2023

Retrospective single-surgeon high-risk cohort: 68 men and 77 cuffs, mean follow-up 32 months. Transcorporal placement was associated with more infection/erosion-related explants (HR 2.74, $P=.03$), including a radiation subgroup association. Surgeons may have chosen it for the most severe tissues, so this cannot prove the technique caused harm. Together with TC20, it prevents a confident universal claim of erosion protection. Abstract checked. [High-risk transcorporal study](#).

ATROPHY20 | Bergeson and colleagues: causes of AUS revision

Selected contemporary reoperative cohort: 137 included revisions/replacements, excluding erosion and absent follow-up. Atrophy was assigned in 11/137 (8.0%) and PRB failure in 47/137 (34.3%). Device era, cuff sizes, surgeon practice and intraoperative definitions affect attribution. The study supports checking mechanics before labelling late leakage as atrophy; it does not show atrophy never occurs or justify routinely using the smallest cuff. Abstract checked. [Revision-cause study](#).

PREP24 | Bourgi and colleagues: AMS 800 skin-preparation protocols

Retrospective sequential-era comparison of 156 procedures using standard versus intensified povidone preparation. Adjusted analysis reported no association with removal (OR 0.97; 95% CI 0.28-3.37). The interval is wide and compatible with substantial benefit or harm. Abstract group counts differ from the table, and reported infection percentages do not reconcile cleanly with counts. Era confounding and follow-up reporting add limitations. Use the qualitative conclusion cautiously; avoid reliable infection-rate/NNT claims or a fifteen-minute-duration conclusion. Relevant full text was checked. [Preparation study](#).

YEUNG13 | Yeung and colleagues: GU prosthetic skin-preparation RCT

Single-centre randomised trial of 100 initial GU prosthetic procedures, 50 per arm. Post-preparation skin cultures were positive in 4/50 with chlorhexidine-alcohol versus 16/50 with povidone-iodine (8% versus 32%; $P=.0091$). The primary endpoint is immediate flora reduction in a mixed-device population. Clinical complications requiring reoperation/removal were few and did not significantly differ. It does not establish lower ContiClassic infection or a preparation duration, and cannot supersede a product's anatomical restrictions. Abstract checked. [Yeung RCT](#).

ABX18 | Adamsky and colleagues: postoperative oral antibiotics

Claims-based observational study, with 3,594 AUS recipients and 10,847 penile-prosthesis recipients. Its endpoint was explantation within three months. AUS explant rates were 3.9% with versus 4.0% without postoperative oral antibiotics, with no association with protection after adjustment. Prescribing was not randomised, claims do not capture all infection details and late events were outside the endpoint. It challenges routine prolonged oral prophylaxis but cannot prove identical safety in every high-risk subgroup. Abstract checked. [Antibiotic study](#).

TOUCH23 | Ziegelmann and colleagues: minimal-touch AUS technique

Historical comparison of a contamination-control bundle with earlier practice, including primary/revision implants. Crude infections appeared lower after introduction, but differences in twelve-month cumulative incidence were not statistically significant for either the overall or primary-implant cohort. Rare events, unequal follow-up and changing practice limit inference. It suggests a sensible potential improvement to study, not proof of a specific glove change, drape or handling pause. Abstract checked. [Minimal-touch study](#).

LINDER15 | Linder and colleagues: long-term primary AMS 800 outcomes

Historical specialist cohort of 1,082 primary implants, median follow-up 4.1 years. Kaplan-Meier freedom from any secondary surgery was 74% at five years and 57% at ten years. The event included infection/erosion, malfunction, attributed atrophy and component-position problems. It informs maintenance/revision counselling. These are not pad-free continence estimates, nor a guarantee of a ten-year service life for an individual device or another manufacturer. Abstract checked. [Linder cohort](#).

AUSCO26 | Kaufman and colleagues: prospective AMS 800 outcomes

Prospective single-arm study at 17 sites, 115 primary implants. At twelve months after activation, 91/97 achieved at least 50% pad-weight improvement and 61/101 reported zero pads; missing outcome denominators differ. Nine participants (7.8% of implanted patients) had revisions in the reported period. Manufacturer funding, expert-centre selection, no control group and one-year follow-up constrain comparative/lifetime inference. It supplies useful contemporary counselling data, not an AMS-versus-ContiClassic comparison. Abstract and indexed primary-results table were checked. Online 2025; 2026 issue. [AUSCO study](#).

Rehearsal prompts and evidence language

- What contraindicates implantation? Distinguish unresolved infection, unsafe storage, inability to empty/use the pump and unstable narrowing. DO alone is different; the actual device label may be more restrictive.
- Which rate do you quote? State the device, population, endpoint, time point and denominator. Prefer a comparable cohort; do not present an early survival percentage as a continence rate.
- Does a non-significant result show no difference? No. It may reflect limited power or imprecision. Equivalence and non-inferiority need prespecified margins and appropriate analyses.
- What does a 95% CI tell you? It describes the range of estimates compatible with the data/model at that confidence level. A wide interval signals uncertainty; inclusion of the no-effect value does not prove equal outcomes.
- What is a hazard ratio? A relative time-to-event rate under a model, not the absolute probability of an event. The definition of event, coding/reference group and time assumptions must be checked.
- Why avoid a universal cuff/catheter rule? Cuff circumference, enclosed tissue and available lumen are different measures. Device state, patient anatomy and current instructions matter.
- What changes your revision plan? Whether leakage reflects use, urethra, mechanics or bladder. Investigate before assuming atrophy or increasing compression.

Scope and source-access note

This is an educational explanation of the supplied deck for an ST5 urologist, with independent checking of its cited sources. It is a critical narrative guide, not a systematic review or a complete operative protocol. All four clinical cases are fictional. Original teaching explanations and speaking lines were written for this guide; articles are linked rather than reproduced.

Relevant BAUS, AUA IPT, ICS AUS and ContiClassic PDF sections, EAU online chapters, [NICE/IDSA](#) recommendations, [AP23](#) text, ChloroPrep product information and [PREP24](#) text were checked. Journal abstracts/metadata were checked via primary publication/index records, including Europe PMC records when PubMed presented an access challenge. Abstract-only limits are labelled in each source entry. The separately referenced ContiClassic OR protocol and current regional/lot applicability remain unverified. This does not constitute clinical or manufacturer approval of the teaching images.

Project provenance: existing repository nityjr-ctrl/aus-presentation, canonical local copy C:\NityProjects\AUS-Presentation\repo, branch manuscript/aus-expert-60min. Prior visual-deck checkpoint 53f2f410bf28de2292f902975d8aac9b797639cb, committed 1 October 2026 at 01:12:51 Europe/London and verified on the configured remote. The attachment was downloaded/modified locally on 5 October 2026 and is explicitly labelled superseded. This guide follows that attachment, rather than assuming a later cloud deck is identical. Repository visibility was verified as public on 5 October 2026; its earlier map entry described the original creation as private.